

BIOSYNC CLINICAL LABORATORY REPORT — URGENT

Patient: Nina Vasquez (DOB: 07/03/1998) | Date: April 14, 2026

Vital Signs & Biometric Readings

Heart Rate	128 BPM
Blood Pressure	98/62 mmHg
SpO2	82%
Temperature	103.2°F
Glucose	338 mg/dL
Weight	101 lbs
BMI	18.4
HRV	14 ms

Clinical Notes

Patient brought in via ambulance by mother Rosa Vasquez. Patient collapsed at home with severe respiratory failure and loss of consciousness. Penicillin allergy documented. Currently on Orkambi, Albuterol, Creon, Azithromycin, Vitamin ADEK, Dornase alfa, IV antibiotics (home course). Heart rate critically elevated at 128 BPM — septic tachycardia. Blood pressure critically low at 98/62 mmHg — septic shock protocol initiated. SpO2 critically low at 82% — patient intubated and mechanically ventilated on arrival. Temperature 103.2°F — high fever, sepsis confirmed. Glucose 338 mg/dL critically elevated — diabetic ketoacidosis secondary to CF-related diabetes and sepsis. Weight 101 lbs — severe nutritional depletion. HRV 14 ms — severely depressed, consistent with septic shock. Blood cultures drawn — broad spectrum antibiotics initiated via port-a-cath. ICU admission confirmed.

Provider Notes

Septic shock with respiratory failure — ICU admission. Patient mechanically ventilated — PEEP optimized for CF lung disease. Vasopressors (norepinephrine) initiated for BP support. Broad spectrum antibiotics: Meropenem and TMP-SMX IV — Pseudomonas and Stenotrophomonas coverage. DKA protocol: insulin drip, aggressive IV fluid resuscitation, electrolyte correction. Pulmonology, endocrinology, infectious disease, and nutrition all consulted. Trikafta genotype results expedited — switch from Orkambi pending confirmation. NG tube placed for enteral nutrition. Mother Rosa Vasquez at bedside — family updated on critical status. Prognosis guarded. Full ICU monitoring in place. Ordering Provider: Dr. Angela Holt, MD — URGENT. Facility: BioSync Clinical Health Center — Medical ICU